



Specimen Collection and Pathology Results Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All clinical services	Medical, Nursing, Allied Health

2. Purpose

To establish minimum practice standards for specimen collection and pathology results throughout Armada Kalamunda Group (AKG).

This procedure should be read in conjunction with the following:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Consent to Treatment Policy OD 0657-16](#)
- East Metro Hospital Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)
- [Clinical Investigations and Results Management Policy AKG-POL-0088-18](#)

3. Background

- The patient's identity must be confirmed prior to the specimen request form being completed, the specimen being taken, and the specimen container label being completed.
- It is mandatory that the request form and specimen container are identically named and verified against patient identification and the exact date and time of collection of the specimen is recorded on the request form and specimen container.
- The collector's declaration box on the specimen request form and the specimen container must be signed or initialled by the collector, this confirms verification of the patient's identification and that the correct specimen is with the correct specimen request form. The signature or initials must match between the form and sample(s).
- Specimen requests that do not conform to the correct labelling requirements or are leaking, cracked, soiled or broken will not be processed.
- It is the responsibility of the requesting health care professional to follow up the results of all specimens taken, inform the patient of these results, take appropriate action on results obtained and document in the patient's health record. If this is not possible during the health care professionals rostered working hours, it should be included as part of clinical handover.
- Samples and request form must be labelled at the time of collection at the patient bedside. Under no circumstances are samples to be taken away from the bedside unlabelled.
- E-Order specimen collections must be done using the e-Order trolley at the patient bedside using the patient wrist band as confirmation of ID.

4. General information

Correct specimen collection, labelling, patient identification and documentation are essential to ensure timely, accurate results that aid in diagnosis and treatment.

Errors in specimen collection or identification of a patient at the time of specimen collection can lead to delays in diagnosis, treatment, misdiagnosis or incorrect treatment resulting in the possible injury or death of a patient.

If uncertain on collection procedures; further instructions on collection and procedures for specific specimens (e.g. Saliva, Blood, Urine, Swabs, Aspirate, Blood Transfusions) may be obtained from [PathWest Test Directory](#).

All locations (except for Theatre) must use electronic pathology ordering, namely eOrder in ED and iCM across all other locations.

5. Procedure

Minimum Patient Identifiers to Confirm Identity

5.1 Infants, children and adults

The minimum mandatory patient identifiers for each patient are:

- FAMILY NAME (upper case) and Given Name/s (title case) in full
- Unique Medical Record Number (UMRN) or equivalent and
- Date of birth

5.2 Neonates

- Samples collected from neonates, before patient identification labels have been created, should be labelled with Mother's ID, and then "Baby of" hand-written clearly on the label, along with the date and time of collection.
- If samples need to be sent to the lab urgently, then send them off. However, it is best to wait until baby's patient identification labels have been printed and placed over the top of mother's label AFTER the mother's label is double-checked and confirmed.
- Where blood group testing is requested, the collector must also sign/initial and date and time both the label on the sample and the declaration on the request form.
- Where blood gas testing is performed on cord or fetal scalp samples, use the maternal ID on the blood gas analyser.

5.3 Confirming patient identity

Before a specimen is collected and the specimen container/s is/are labelled, the collector of the specimen must verify the identity of the patient by:

- Asking the patient (if conscious and able) to spell their family and given names and state their date of birth. Where necessary, next of kin/legal guardian/carer may provide this information.
- Checking the stated minimum patient identifiers against the request form and the patient's identification band;
- If the patient is not wearing an identification band refer to site specific guidelines.

Prior to the specimen being taken the health care professional must obtain consent from the patient or next of kin/legal guardian/carer. Refer to [WA Health Consent to Treatment Policy 2016 OD 0657/16](#)

5.4 Where patient's identity cannot be reliably confirmed

When a patient's identity cannot be reliably confirmed (e.g. patient is unconscious, intoxicated, mentally impaired, or experiencing language difficulties) they must be registered as 'Unknown Male' or 'Unknown Female' using an emergency UMRN.

Pre-printed 'Unknown' files are recommended for emergency patients for resuscitation where samples/investigations must be initiated prior to patient registration.

- Once the patient is identified, patient information should be updated and a new identification band attached. Local operational policy must stipulate procedures to ensure that such patients can be correctly identified throughout their admission; particularly, in relation to the reconciliation of samples/investigations using either UMRN, with the right patient.

If a previous UMRN is found, the pre-existing medical record will be merged with the new medical record. Refer to [EMHS Patient Identification and Procedure Matching Policy](#).

5.5 Completion of specimen request forms

- An electronic request form via CPOE (electronic ordering) must accompany all specimens and the health care professional must detail the specific tests required, provide relevant history/reason for the test, sign the request form and provide a current and valid provider number. CPOE downtime form should only be utilised if CPOE (electronic ordering) is not available.
- The health care professional must confirm the patient's identity prior to completing the request form. If further tests are required from the same specimen, a new request form must be completed and forwarded to Pathology Department (CPOE (electronic ordering) is preferred but a written CPOE downtime form may be acceptable for additional test requests).
- E-Order must be used in place of CPOE in the Emergency Department.
- Written Request forms must include the minimum patient identifiers and information below, Consultant name, be legible and written in indelible pen. If information is inadequate/insufficient, the specimen is to be referred to the requesting Doctor to complete the relevant details.
- If either the request or requestor are not legible, Pathwest will contact treating team for clarification and further details.

5.6 Minimum patient identifiers to confirm identity

- Patients full name (Surname and Given Name)
- Unique Medical Record Number (UMRN)
- Date of Birth
- Patient Location (if hospital in-patient)
- Clinical Diagnosis
- Test required written by requesting health care professional
- Requesting health care professional's printed name, signature and provider number
- Type of specimen

- Date and Time of Collection
- Signature of specimen collector

5.7 Collection of specimens

All specimens will be collected in the appropriate leak-proof container and transported in a sealed, leak-proof biological hazard bag with the request form in a separate compartment.

Specimens must be promptly transported to the laboratory or stored in a designated specimen fridge until transport is arranged, unless otherwise directed.

Specimens without appropriately completed forms will not be collected or transported to Pathology.

5.8 Equipment required

Prior to collection of specimens the collector must ensure they have:

- Appropriate personal protective equipment dependant on specimen required
- Appropriate specimen containers for specimen required
- The most appropriate equipment required to obtain specimen must used
- Any equipment required must be serviced and calibrated in accordance with manufacturer's recommendations to ensure reliability and accuracy
- Consult the online PathWest Specimen Collection Directory for sample collection information: <http://pathlines.health.wa.gov.au/testdirectory/>

5.9 Specimen container labelling

- Specimen containers must not be labelled before the specimen is collected but **labelled immediately after** the specimen has been collected and placed in the container.
- Before labelling the specimen container/s the collector must confirm the patients identify, refer to [Confirming patient identity](#) and check against both the patient identification band and the specimen request form.
- It is mandatory for the minimum patient identifiers and collector's signature to be included on the specimen container label and that both the request form and sample container are identically named and verified against the patient's identification band. The site the specimen was taken from must also be noted on the label.
- Labels may be hand written or a patient identification label used as long as the minimum patient identifiers are included. Hand written labels must be legible and written in indelible pen.
- The exact date and time of collection of the specimen must be recorded on the request form and the sample container label.
- The collector's declaration box on the specimen request form and the specimen container must be signed or initialled (matching) by the collector, this confirms verification of the patient's identification and that the correct specimen is with the correct specimen request form.

5.10 Unlabelled, mislabelled or damaged specimens

Where due to the nature or site of the sample repeat specimen collection cannot be readily undertaken, i.e. tissue biopsy or any other sample collected by invasive procedure, the

PathWest Senior Medical Scientist (or equivalent) of the relevant SMHS or WACHS site in discussion with the patient's treating consultant will determine if the specimen will be processed.

The following will not be processed under any circumstances:-

- Unlabelled/mislabelled **transfusion medicine**
- Specimens not labelled with the minimum patient identifiers
- Where the details on the specimen container are not identical to those on the specimen request form
- Specimens received in leaking, cracked, soiled or broken specimen containers

For further information on minimum requirements for clinical samples and request forms refer to, [PathWest](#).

6. Wound swab for microbiology

Wound swab or pus collection for microscopy and culture are indicated for moderate or severe infection, especially when there is spreading cellulitis or symptoms and signs of systemic infection.

Kit contents and processing information is on the [PathWest intranet site](#) (Search "Wound swab" and select "Wound swab – MC&S")

6.1 Equipment

- PathWest bacteriology general swab kit
- Normal saline
- Unsterile gloves
- Completed microbiology request form documenting current topical or antibiotic therapy if any
- Biohazard bag

6.2 Key points

- Observe Standard Precautions throughout procedure
- The presence of slough or necrotic tissue is not an indication for a swab
- Aspiration of pus is preferable to a swab of pus or wound swab
- Wound swabs should be collected before antibiotics are commenced
- Tissue, pus and fresh exudate are the preferred specimens
- Wound cleansing reduces the risk of false positives. Do not use antiseptic
- For surgical wounds, deep wounds, diabetic foot wounds, if osteomyelitis is suspected then sample the most inflamed area. It may be necessary to probe and perform a deep swab to capture anaerobes
- Where there is doubt about infection, collecting swabs for some clinical presentations is unnecessary i.e. penile discharge, PEG sites, pressure injuries and non-healing ulcers
- For wounds of an atypical nature, consider aspirates, biopsies or alternative approaches
- Explain the procedure and confirm consent from the patient before commencing
- Ensure the details on the patient ID band and CPOE form match

6.3 Process

1. Aspirate pus using a large bore needle and syringe
2. Carefully syringe pus into specimen container

OR

1. Cleanse wound by gently swabbing or irrigating with Normal Saline to remove surface debris and dressing residue
2. Wait 1-2 minutes then following instructions on swabbing kit take swabs from viable tissue, not eschar or slough
3. Use the transport swab to swab pus and insert the swab into the charcoal transport medium provided
4. Using the sterile dry swab, sample the wound again and make a smear in the middle the frosted side of the glass, in a circular motion approximately 1cm x 1cm diameter
5. Label with slide with patient's name, date of birth, date and time of collection and type of specimen e.g. wound location, side and exact position
6. Ensure clinical history on form includes clinical problem e.g. "oozing post op hernia repair wound with cellulitis" and not simply "wound swab"
7. Place specimens in Biohazard bag with request form in separate pocket and send set to PathWest Laboratory
8. Document in the patient's integrated notes

7. Urine and faecal specimen collection

7.1 Background

- Adhere to Standard Precautions throughout the procedures.
- Inform patient of procedure to be performed and obtain informed consent.
- Genital cleansing is generally ineffective at reducing contamination however cleansing may be necessary if the patient is incontinent of urine and/or faeces or has poor hygiene.
- For optimal mid-stream urine specimen collection, collect the second urine for the day unless specific collection times are required.
- For catheter specimen urine collection, the indwelling catheter is to be less than 3 days old, if not, liaise with medical staff as indwelling catheter will require changing before collection CSU can occur.
- Label container with patient identification label, date and initial sticker.
- If the patient is currently taking or recently ceased antibiotics note this on the request form.
- For any non MC&S collections refer to [PathWest search engine](#)

7.2 Equipment for urine collection

- Yellow top sterile container
- Urinal or bedpan
- Normal saline and gauze (optional)
- Biohazard bag
- Personal protective equipment (PPE)

7.3 Process for mid-stream urine collection

1. Assess the level of assistance the patient requires. Males - if genital cleansing is necessary and the patient is unable to wash with soap and water this should be done with normal saline swabs. If the patient is uncircumcised, retract the foreskin and clean meatal area with a circular motion. The foreskin should remain retracted during urine collection. Females – if genital cleansing is required, wash front to back with soap and water, then rinse with water before drying.
2. Females should be instructed to retract the labia while urinating.
3. Instruct both male and female patients to forcibly void a small amount into toilet, urinal or bedpan and then momentarily stop urine flow. The specimen container should be positioned to catch the mid portion of urine without contaminating the container. If patient is unable to ambulate to toilet, collect mid portion of urine directly into specimen jar. NB: It is preferable to collect the urine into a sterile container however to avoid patient embarrassment a clean container is acceptable.
4. Instruct the patient to complete voiding into the toilet, urinal or bedpan.
5. Full foreskin back into normal position if required.
6. Transfer urine to specimen jar if necessary, not filling more than half full.
7. Transfer correctly labelled specimen into Biohazard bag with signed request form. Send to Pathology within 30 minutes or refrigerate until able to send to Lab.
8. Document specimen collection in integrated notes.

7.4 Process for catheter specimen of urine

Additional equipment needed:

- 10ml syringe
 - Alcohol swab
 - Personal protective equipment (PPE)
1. Swab sampling port with alcohol swab. Allow to dry.
 2. Insert syringe and withdraw specimen – 10ml is required.
 3. Remove syringe from sampling port.
 4. Transfer urine into sterile yellow top container.
 5. Dispose of syringe.
 6. Transfer specimen into Biohazard bag with request form. Send to Pathology.
 7. Document specimen collection in integrated notes.

7.5 Equipment for faecal collection

- Sterile white container with brown top with scoop attached for faeces
- Biohazard bag
- Personal protective equipment

7.6 Process faecal collection

1. Using a freshly sanitized pan. Use a wooden spoon or scoop on lid of brown top jar to transfer specimen to the container.
2. Discard disposables and decontaminate remaining equipment

3. Transfer specimen into Biohazard bag with request form. Send to Pathology within 30minutes of collection.
4. Document in integrated notes specimen collection.

8. Theatre/Endoscopy Specimens

- Collection of a segment of body tissue or fluid for the purpose of pathological, cytological or histological investigation is the decision of the clinician responsible for the procedure
- Specimen handling shall be assessed and planned prior to the commencement of the scheduled procedure/list.
- Correct patient and specimen identification shall be confirmed to minimise the risk of an adverse outcome for the patient.
- The perioperative nurse has a duty to ensure that correct collection and handling methods are implemented for the protection of the specimen.
- Specimen containers shall be accurately labelled ensuring they match the pathology request form.
- Documentation methods shall be established to ensure specimen accuracy and accountability.
- Transfer and transport methods shall be established to ensure the integrity of the specimen/s.
- If the specimen is large enough to require a container that is non pre-filled with transport medium, it should be treated as a fresh specimen and sent to pathology for the decanting of formalin onto the specimen.
- Fresh specimens are to be delivered to pathology on completion of the case. This will be documented in the pathology register along with the time of delivery and the name of the staff member undertaking the delivery.

Role	Responsibility
Instrument (Scrub) nurse	• The required segment of the body tissue or fluid is collected and deposited in the correct specimen container with the relevant transport medium in consultation with the operating specialist. • The specimen details and correct labelling are confirmed before specimen is placed in the specimen container.
Circulating (Scout) nurse	• The container and pathology request form are correctly labelled with the patient's name, unit number, date/time of collection and description of specimen. All documentation is completed legibly and correctly. • The patient sticker is to be placed on the body of the container, not the lid. • The required segment of the body tissue or fluid is collected and deposited in the correct specimen container with the relevant transport medium. Note – specimens being transported in formalin must be placed in a suitable size container to ensure there is at least twice the amount of formalin to the size of the specimen.

Role	Responsibility
	- The specimen details and correct labelling are confirmed before specimen is placed in the specimen container. - Specimen details are entered on TMS (Theatre Management System) ensuring they are entered under the correct procedure if multiple procedures are being performed. - Specimen container is placed in a “hazard” bag for collection with the completed request form, ensuring documentation on request form matches specimen. - The specimen is recorded in the pathology register and placed in the pathology specimen collection tray after a last check to ensure the correct patient and specimen details are documented.
Pathology personnel	The specimen and request form, are collected by the pathology personnel, ensuring matching documentation.

9. Results

Refer to the [Clinical Investigations and Results Management Policy AKG-POL-0088-18](#)

It is the responsibility of the requesting health care professional to follow up the results of all specimens taken, inform the patient of these results, initiate appropriate action based on the results obtained and document in the patient's health record. If this is not possible during the health care professionals rostered working hours, it should be included as part of clinical handover, refer to [WA Health Clinical Handover Policy MP 0095](#).

10. Documentation

All specimens taken must be recorded in the patient's health record, detailing date, time and type of specimen taken and the results of all specimens and that the patient has been advised of the results and appropriate action taken on results.

Failure to accurately and legibly record, and understand what is recorded, in patient health records contribute to a decrease in the quality and safety of patient care.

11. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Consent to Treatment Policy OD 0657-16](#)
- East Metro Hospital Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)

12. Supporting information

The following documents support the implementation of this procedure:

- [Clinical Investigations and Results Management Policy AKG-POL-0088-18](#)

- [Clinical Handover Procedure AKG-PRO-0521-19](#)
- [Identification of Patients in the Armadale Mental Health Service MEN-PRO-0203-14](#)
- ACORN Australian College of Operating Room Nurses (ACORN), (2018). Standards for Perioperative Nursing in Australia (15th ed.) Adelaide, South Australia: ACORN

13. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

Organisation-wide data trends and risks associated with specimen collection are to be reported to, and monitored by, the Clinical Outcomes Committee.

14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.5 Healthcare rights and informed consent
- The health service organisation has processes to identify:
 - a. The capacity of a patient to make decisions about their own care
 - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves
- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
 - a. Defines approved identifiers for patients according to best-practice guidelines
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover
- Standard 6 – Action 6.11 Documentation of information
- The health service organisation has processes to contemporaneously document information in the healthcare record, including:
 - a. Critical information, alerts and risks
 - b. Reassessment processes and outcomes
 - c. Changes to the care plan

15. Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Director Clinical Services
Department: Armadale Kalamunda Group
Email: tony.robins@health.wa.gov.au

16. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 3.2	05/12/2022	05/12/2025	Minor amendment

17. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Alison Parr – Director Clinical Services
Authorisation date	01/12/2022
Published date	05/12/2022